B. Recommended for favorable prognosis is the use of expanded day 5 or 6 blastocysts with well-defined inner-cell mass and trophectoderm as defined by the individual embryology laboratory.

When treatment for infertility is not covered:

A. Gamete intrafallopian transfer (GIFT) and zygote intrafallopian transfer (ZIFT)
B. Multiple Embryo Transfer
C. Charges for gestational carrier or surrogacy, including antenatal appointments and labor/delivery services
D. Charges for procedures to collect, analyze, manipulate, or otherwise treat gametes (sperm and ova) when the partner or donor who produces the gamete is not a covered member on the plan
E. Cost of donor egg
F. Cost of donor sperm
G. Elective preservation, such as egg freezing sought due to natural aging
H. Fertility drugs, provided by facilities or physicians, including ovulation induction cycles while on injectable medication to stimulate the ovaries. Fertility drugs must be obtained through the pharmacy benefits, see Prescription Drug Benefits and Specialty Drug Benefits within your plan brochure. Medications will not be covered when dispensed by other sources, including physician offices, home health agencies and outpatient hospitals.
I. Genetic counseling
J. Infertility services after voluntary sterilizations
K. Reversal of voluntary surgical sterilizations
L. Services and supplies related to non-covered ART procedures.
M. Treatments such as artificial insemination, assisted reproductive technology, and/or in vitro fertilization prior to establishing diagnosis of infertility.

The following are unproven and not medically necessary for diagnosing and/or treatment of infertility:

A. Co-culture of embryos
B. Computer assisted sperm analysis (CASA)
C. Cryopreservation of immature oocytes, ovarian tissue or testicular tissue
D. EmbryoGlue
E. Hyaluronan binding assay (HBA)
F. In vitro maturation (IVM) of oocytes
G. Inhibin B
H. Postcoital cervical mucus penetration test
I. Reactive oxygen species (ROS) testing
J. Sperm Acrosome reaction test